

Lakeside Medical Center - Gastroenterology Department

1234 Health Drive, Medtown, State 12345 | Phone: (555) 678-9012 | Fax: (555) 678-9013

Dr. Emily Carson, MD - Attending Gastroenterologist

Registration / Demographics

Patient Name:	Johnathan Edwards	DOB:	Oct 2, 1975
Age:	48	Sex:	Male
Address:	789 Pine St, Medtown, State 12345	Phone:	(555) 345-6789
MRN:	MRN123456	Acct:	ACCT789012
Insurance:	HealthPlus Plan B	Emergency Contact:	Jane Edwards (555) 987-6543
Encounter Date:	Sep 21, 2023	Check-in Time:	08:15 AM
Location:	Endoscopy Suite 3	Referring Provider:	Dr. Marcus Lane, MD
Chief Complaint:	Dyspepsia	Indication:	GERD evaluation

Patient Medical History

PMH: Chronic GERD, Hypertension, Hyperlipidemia. PSH: Appendectomy (1990). FHx: Father - CAD; Mother - Type 2 Diabetes. SHx: Non-smoker, occasional social alcohol. Allergies: Penicillin (rash). Problem List: GERD, Hypertension, Anxiety.

Current Medications

- Omeprazole 20mg daily
- Lisinopril 10mg daily
- Atorvastatin 20mg nightly
- Alprazolam 0.5mg prn anxiety
- Aspirin 81mg daily

Pre-Procedure Vitals

BP: 120/80 mmHg, HR: 72 bpm, SpO2: 99% on room air. ASA Classification: II.

Informed Consent

The patient was informed of the risks and benefits of the procedure, including potential complications. Questions were addressed, and informed consent was obtained and documented.

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Pre-Procedure Assessment & Anesthesia Record

Pre-Anesthesia Evaluation

Airway: Mallampati II. Cardio: Regular rhythm, no murmurs. Pulm: Clear to auscultation bilaterally. NPO Status: Confirmed 8 hours. Last intake: 10:00 PM previous night.

Planned Sedation

Moderate sedation planned with midazolam 2mg IV and fentanyl 50mcg IV.

Anesthesia Team

Anesthesiologist: Dr. Martin Bower, DO. CRNA: Amelia Thompson, CRNA.

Procedure Timeline

Event	Time
Anesthesia Start	09:00 AM
Procedure Start (Scope In)	09:10 AM
Procedure End (Scope Out)	09:40 AM
Anesthesia End	09:45 AM
Recovery Start	09:50 AM
Recovery End	10:30 AM

Monitoring Trends

Time	BP (mmHg)	HR (bpm)	SpO2 (%)
09:00	122/78	74	99
09:20	118/77	72	98
09:40	116/75	70	98

Complications

No significant complications. Brief desaturation to 92% resolved with O2 adjustment and jaw thrust. No further events noted.

Post-Anesthesia Evaluation

Aldrete score: 9. Disposition: Discharged home with escort.

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Procedure Note: Esophagogastroduodenoscopy (EGD)

Procedure Details

Endoscope Model: ScopeCo Model 100. Route: Oropharyngeal entry, UES passage. Patient Position: Left lateral decubitus. Bite Block: Applied.

Narrative

The scope traversed the oropharynx and comfortably passed through the UES. The esophagus was visualized, notable for a normal Z-line at 40cm. GEJ at 45cm from incisors. Mild erythema noted, no rings or strictures apparent. The stomach was entered, revealing the body, fundus, and antrum in good view with insufflation. Rugal folds were normal. Mild non-erosive gastritis observed in the gastric body. Pylorus open and easily passable. Entered duodenum, bulb and second portion visualized, villi intact. No ulcers or active bleeding seen. Retroflexion in the stomach to assess cardia/fundus was unremarkable.

Findings

- Mild erythema at the GEJ
- Non-erosive gastritis in gastric body

Impression

Mild GERD changes. Non-erosive gastritis.

Diagnosis

- Gastroesophageal Reflux Disease (GERD)
- Non-Erosive Gastritis

Interventions

No biopsies or interventions performed during this procedure.

Endoscopy Team

Endoscopist: Dr. Emily Carson, MD. Assistant: Sarah Gleason, RN Circulator.
Endoscopy Tech: Lucas Dean.

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Specimens / Lab / Pathology

No specimens collected. No biopsies were taken during this procedure.

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After Visit Summary / Discharge Instructions

Recovery Course

The patient was stable post-procedure, with minimal discomfort and no nausea reported. Vitals remained stable throughout recovery.

Diet and Activity Instructions

Resume regular diet as tolerated. Avoid heavy or greasy foods for the next 24 hours. Rest and limit physical activity for the rest of the day.

Warning Signs

Seek medical attention for severe abdominal pain, persistent vomiting, or signs of bleeding (black/tarry stools).

Medication and Follow-Up Plan

Continue current medications. Start pantoprazole 40mg daily for improved GERD management. Follow up with Dr. Carson in 4 weeks.

Results Communication Plan

All findings and recommendations were communicated before discharge. Next appointment scheduled for Oct 20, 2023.

Patient Acknowledgment

The patient verbally acknowledged understanding of the discharge instructions and demonstrated the ability to recall key points.

Escort and Transport

Discharged with wife, who will drive him home.

RN Discharge Educator

Sarah Gleason, RN - 10:40 AM

Provider Signature

Dr. Emily Carson, MD - 10:45 AM | Electronically signed by Dr. Emily Carson

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